• • • REFLECTIVE PIECE

The Institution of the Mind – The Final Challenge

ABSTRACT

Deinstitutionalisation is a phenomenon with extensive practical consequences, as it leads to changes in the lives of people with a disability, their families and staff. It has also extensive theoretical consequences for disability services, as new models are needed when forming alternative services to those offered by the institution. During a deinstitutionalisation project (Ericsson, 2002) practical as well as theoretical problems were faced. One of the unexpected phenomena we met with came to be termed 'the institution of the mind'.

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FROM INSTITUTION- TO COMMUNITY-BASED SERVICES

The basic assumption was that people who live in a residential institution are in need of services because of their disability. The decision to close the institution expresses no denial of this fact. What is necessary, however, are services relevant to their needs, delivered in non-institutional settings. If people in the institution are provided with such community-based services, they can move and the institution can close. The key to deinstitutionalisation therefore lies in the development of community-based services. A general service model has to be formed and services have to be arranged for each person.

The model for community-based services grew out of Swedish experience. It comprised two basic services, the group home and the day activity centre. The home and a setting for activities, in different places, became the platform from which the person could participate in community life. Relations could, for example, be established with commercial life, family and social life, and recreation, leisure and welfare services. The group home is a house for a group of people who live there, while employed staff provide them with the support they need. In practice, because people with varying needs move out of the institution, group homes also need to vary. Changes may have to be made to the physical form of a house. The size of the group living there can vary, as may the training and size of the staff group. The same approach applies to the activity group, which comprises a small group of people who spend their days together, participating in a number of activities.

Based on an assessment of people's needs, varieties of group homes and places for daily activities were established, making it possible for

all to move, and so the institution was closed. A new structure of services was created with a higher standard of living. As these were organised so that people could live in small groups, more personal relationships could develop between the people in a group and between people and staff. Services were closer to the life of the community, as group homes were located in residential areas and daily activities in places for work. Generally, a better life was offered than that which had been provided in the institution. It must also be said that not all new opportunities were taken up; more could have been done to enhance the disability services and the way of life they offered.

Based on an analysis of life in the institution, it was hoped that there would be an increase in equality and respect in social relations between people and staff in the new services. This could lead to the person's experiencing more influence over his everyday life, based on requests expressed by himself or through his representative. There were also expectations that the new services would be receptive to the involvement of families. Proximity to community life also raised expectations of more extensive use of local resources. However, this occurred to only a limited extent, leading to frustration and comparisons with life in the institution. The lack of achievement in these respects gave rise to the expression 'there is an institution in one's mind'.

TWO TRADITIONS OF SUPPORT

The normalisation principle of 1946

A search for the roots of deinstitutionalisation in Sweden leads back to the 1940s, when a national welfare society was being developed. Naturally, people with a disability, through their organisations, were anxious to know how their need for welfare was to be met. The only services available to them at this time were the existing old residential institutions. This issue was brought to attention in a public inquiry whose committee delivered its suggestions in 1946 (SOU, 1946:24). The proposals were concerned with all forms of disability, but they made suggestions only for those with a limited need for support, those at the time called 'partially able-bodied'.

This committee formulated a vision of how to satisfy the needs for welfare of people with a disability. Instead of choosing to modernise old institutions, they advocated an alternative, in which the new welfare services were to be given responsibility. This required that they extend their competence to meet the needs of people with a disability. The committee based this suggestion on recognition of these people as full citizens, belonging to society. As citizens they would have the democratic right to use public welfare services. If they were to be used, a person could live in the community to which he and his family belonged, where he could lead an everyday life like others in more normal circumstances. This idea, formulated by the committee, was called the normalisation principle.

This new socio-political idea was the starting point for the Swedish disability reform. To increase the participation of people in the welfare society, community-based services had to be developed. As the normalisation principle was based on a critique of residential institutions, this was also the beginning of the dissolution of institutional services.

Development and dissolution

The expression 'partially able-bodied', used by the committee, is an illustration of a view that separates those with limited need for support from those with more extensive needs. As this was the dominating view during the '50s and '60s, the first steps towards a transition of services was concerned with the development of community-based services only for those with limited need for support. Others were seen as being in need of services from an institution. During this period, social innovations led to new forms of service, such as non-institutional education, community-based group homes and day activity centres. Institutions were still seen as desirable.

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During the '70s, provisions offering a more normal life, a higher material standard and a higher degree of participation in community life became the preferred service. As a consequence, the institutional critique became more outspoken and was recognised formally in the directives for the inquiry of 1977, which clarified that residential institutions should be closed. During the period 1975–2000 all residential institutions were closed and the residential institution was no longer accepted as a disability service.

A shift between two traditions of support

The normalisation principle of 1946 gave rise to a wider frame of reference within which to understand the deinstitutionalisation process. The community tradition recognises people with a disability first and foremost as citizens, with rights like other citizens. Participation in community life becomes natural, together with the use of public welfare services. The context for this community tradition is found in the society of the 1940s, when issues like democracy, welfare and human rights, the latter expressed in the UN *Declaration of Human Rights* (United Nations, 1948), were current.

With a citizen perspective, this tradition focuses on the quality of everyday life of a person with a disability and his request for welfare and a good life; he has the same needs as other citizens. The content of everyday life which is desired becomes a very personal matter, expressed by the person, or if necessary through his representative. Services required to contribute to a good life are the result of a process, where one party is the provider of services and the other is the person in need of support. If no agreement can be found, both parties can appeal to the court for a decision on a suitable service (SFS, 1993:387). Democracy and the possibility of influencing one's own life are inherent aspects of this tradition. Community participation becomes a right for people, realised only when adequate services are provided.

The institutional tradition has its roots in the second half of 1800, when industrialisation made people with a disability visible and led to their being seen as a problem of care for society. To respond to these needs, society set up a number of institutions, each with content considered relevant for a certain form of disability: institutions for the blind, the deaf, the epileptic, those with psychiatric problems and people with intellectual disability (learning disability). When people were taken care of, they were characterised by their disability and placed in the institution seen as relevant.

There were basically two types of special institution for people with learning disability, one for those seen as educable and another for those seen as uneducable. The social roles of pupils and patients were thereby attributed to them: two roles which express that these were people lacking in competence. A competence perspective is therefore part of the institutional tradition; the task of the institution was to increase the competence of people to such a degree that they could leave when able to meet the demands made by the outside world. Community participation becomes a reality in this tradition when the person has acquired this level of competence.

The shift between these two traditions therefore takes place on three levels. At individual level, leaving an institutional life for community participation becomes the objective. At organisational level, dissolving special institutions becomes a task which runs hand in hand with making public welfare services available. At the cultural level, a citizen perspective needs to be captured, as the competence perspective is discarded.

SETTING UP NEW LIVES

Historically, the community tradition acquired its character from the normalisation principle of 1946, recognising the full citizenship of people with a disability. Today, this view is strengthened by the human rights approach which has been gradually developed by the UN since 1948. The core value of this tradition is the right of the person to participate in creating his everyday life. Setting up lives then becomes the key task.

A new life is of course a matter for the person himself, whose opinion, if necessary, is expressed

through his family and his representative. Staff in the home and in daily activities in the community are naturally part of this process. There are also local welfare organisations involved. This evolves into a process in which a number of bodies are contributors.

It has been necessary and natural to apply this approach when people who have grown up in their family home, as adults want to move to a home of their own. People have a good knowledge of the conditions of a normal life; they know what a house and a home is like and what to expect from employment. To meet their requests it is not possible to offer special services, based on national norms or models. Instead it is important to be able to make use of local resources. The person may not ask for a special group home, but instead a rather insignificant house in the village where he lives. His requests for daily activities may be to spend the days of the week in places where he is known and feels welcome. In order to make use of local resources in this way it is necessary to have people with knowledge of the local community involved in setting up the person's new life.

During deinstitutionalisation, when a number of people leave the residential institution, the task is to identify those people who will assist the person in setting up his new life. Representatives from the residential institution can contribute in this process, but setting up a new life in the community is not the task of the institution.

THE INSTITUTION OF THE MIND

Deinstitutionalisation must be recognised as a major issue, as it concerns a change from a life in a residential institution to participation in community life for people with learning disability. Experiences from the project discussed here illustrate, however, that even though it was successful, succeeding in closing and replacing the institution with new disability services, there were further problems that had to be solved. The expression 'institution of the mind' illustrates this. The rather unspecific concept of deinstitutionalisation needed to be clarified to give this expression meaning.

The frame of reference used in the analysis of this deinstitutionalisation project, two traditions of support, gave a wider view of the issues involved. It was not enough to replace the institution with new special services – this merely provided a further reason for continuing to relate to people in the old way. To understand fully the shift between traditions, attention had to be paid to the role of citizenship, recognising people's democratic right to participate in forming their own life. The expression 'the institution of the mind' was formulated because it was observed that the only change which had taken place was a move from a special institution to special disability services, even if they were in the community. The frame of reference proposed also provides guidance on how to close the institution of the mind. An initial shift to a community tradition has to take place, and this requires acceptance and recognition of people as full citizens.

How does one capture a citizen perspective? This question is not rhetorical, but very real. At present it is hard to find activities which enable those concerned to see the citizenship of people. This must therefore be a major task to solve during deinstitutionalisation.

There are some experiences, though limited, from a series of projects. The basic assumption behind them has been that the democratic character of citizenship presupposes a relationship of equality between the person with disability and those providing services. This exists on a formal level, as the person can have his rights to the services offered tested in court. What is needed, however, is a formal and democratic forum within the services, where the person is given an opportunity to influence the content of his everyday life. A limited project concerning this issue showed that it had profound consequences for people, as well as for staff (Ericsson & Ericsson, 2004). New relationships were created, enabling people to acquire the role of citizen. Staff also changed, seeing people to a higher degree as citizens.

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